



Direct: 703-621-3359

Main: 888-224-7262

Fax: 703-621-3356

February 1, 2026

Mr. Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Attention: HHS Health Section AI RFI
330 C Street S.W.
Washington DC 20201

Re: HHS Health Sector AI RFI

Dr. Keane:

The Radiology Business Management Association (RBMA) appreciates the opportunity to respond to the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence.

Established in 1968, RBMA is a professional association that consists of over 2000 radiology practice business leaders who represent over 800 radiology practices in all 50 states. Included are diagnostic radiology, interventional radiology, nuclear medicine, Independent Diagnostic Testing Facilities (IDTFs) and radiation oncology.

RBMA is the trusted partner of radiology professionals, advancing the industry and broadening our members' capacity to provide superior patient experiences.

RBMA represents the "business side of radiology." While our members are deeply familiar with the clinical benefits and challenges of AI, our responses focus primarily on the financial, operational, and administrative implications of AI adoption. We defer to physician organizations on detailed clinical benefits and barriers.

1. What are the biggest barriers to private sector innovation for AI for healthcare and its adoption and use in clinical care?

RBMA Comments:

Significant barriers are **financial**, **operational**, and **contractual** in nature.

- **Financial:** Organizations struggle to determine how to pay for AI, how reimbursement will evolve, and how to build viable financial models while AI use cases remain fluid.
- **Operational:** Integrating AI into established workflows can be complex. Workforce readiness and willingness to adopt AI tools are still uncertain.
- **Contractual:** Key liability questions remain unresolved: Who is responsible when AI makes an error? How will malpractice insurers treat AI-assisted care? Additionally, concerns exist regarding **data ownership**, vendor data governance, and whether vendors intend to **monetize clinical data**.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to

incentivize the effective use of AI in clinical care and why?

RBMA Comments:

HHS should not assume AI will immediately create efficiencies. Early implementation often requires **increased** workflow steps, staff time, and capital investment. If HHS wishes to incentivize AI adoption, it should pursue approaches that do **not** reduce base reimbursement and instead:

- Support clinicians and providers with **funding mechanisms** for procurement, implementation, and ongoing operationalization.
- Create and enforce regulations that clarify **ownership of data, liability standards, and malpractice coverage expectations** related to AI use.
- Establish a **federal certification framework** verifying AI vendor compliance with, and ideally exceeding, HIPAA and cybersecurity standards.
- Ensure that AI tools and technologies—where applicable—are **exempt from tariffs**, reducing financial burden and promoting access.

Clinicians remain unsure how to model the long-term costs and reimbursement of AI; HHS policies that reduce cost barriers will significantly improve adoption.

3. For non-medical devices, what novel legal and implementation issues exist, and what role should HHS play?

RBMA Comments:

RBMA agrees that significant legal uncertainties exist—many of which affect both medical and non-medical AI tools. As noted in response to Question 1, practices face unresolved questions related to:

- HIPAA and broader privacy compliance
- Malpractice and legal liability
- Data ownership, governance, and vendor rights to reuse or monetize data

HHS can help accelerate adoption by establishing clearer guardrails, reducing ambiguity for providers, and creating trusted standards for vendors.

4. For non-medical devices, what are the most promising AI evaluation methods, and should HHS support these processes?

RBMA Comments:

RBMA recommends that HHS support evaluation methods that promote **transparency, robustness, and workflow safety**, including:

- Pre-deployment validation using diverse, representative datasets
- Post-deployment monitoring for model drift, error rates, and performance across demographic groups
- Human-centered workflow assessment to evaluate the real-world impact on clinical staff and operations

HHS could accelerate progress through **grants, cooperative agreements, or vendor certification competitions** that reward reliability, equity, and interoperability.

5. How can HHS best support private-sector activities (e.g., accreditation, certification, credentialing) to promote AI use?

RBMA Comments:

HHS can play a critical role by:

- Establishing **baseline national standards** for AI vendor transparency, cybersecurity, data governance, and model training disclosures

- Partnering with private-sector accreditation bodies to create **aligned, non-duplicative certification pathways**
- Encouraging industry-driven benchmarking tools that incorporate privacy, equity, and workflow-impact criteria

Such efforts would reduce provider burden and strengthen trust in AI tools.

6. Where has AI in clinical care met or exceeded expectations, and where has it fallen short? What novel tools hold the most promise?

RBMA Comments:

RBMA's expertise centers on the business and operational aspects of radiology. However, from a clinical lens, RBMA notes ongoing concerns regarding **bias in AI training datasets**. AI models learn from the data they ingest, and many tools may not be validated against sufficiently diverse patient populations.

Examples include:

- **AI in mammography:** Were models trained on women across all breast density categories, varied risk profiles, and diverse racial and ethnic backgrounds?
- **AI in pneumonia detection:** Were tools trained solely on acutely ill hospitalized patients, or do they include routine outpatient imaging?

Similar to the HIPAA-oriented certification noted in Question 2, HHS could significantly support providers by creating a **federal certification verifying that AI models were trained and validated on comprehensive, demographically diverse datasets**.

7. Which decision makers drive AI adoption within healthcare organizations, and what are the administrative hurdles?

RBMA Comments:

Key decision makers typically include executive leaders (CFO, CIO, CMO), radiology administrators, IT departments, compliance officers, and clinical governance committees. Primary hurdles include:

- Limited capital budgets
 - Variable reimbursement and uncertain ROI
 - Integration challenges with existing systems
 - Legal and compliance ambiguity
 - Staff acceptance and training needs
-

8. Where would enhance interoperability widen AI market opportunities and accelerate development?

RBMA Comments:

Enhanced interoperability—particularly around imaging data formats, structured reporting, and standardized benchmarking data—could dramatically advance AI by allowing:

- Easier dataset aggregation for model training
- Seamless integration with PACS/RIS/EHR systems
- More reliable cross-vendor comparisons
- Faster multi-site validation studies

Standards such as DICOM, HL7/FHIR, and imaging-specific structured reporting frameworks are essential.

9. What challenges do patients and caregivers want AI to address, and what concerns do they hold?

RBMA Comments:

Patients express interest in AI tools that improve **access, accuracy, timeliness, and care coordination**. However, they also express concerns related to:

- Data privacy and security
- Equity and bias in AI decision-making
- Whether clinicians remain “in the loop”
- Transparency around how AI is used in their care

Clear communication from providers—supported by standardized HHS guidance—will be essential for patient trust.

10. Are there AI research areas HHS should prioritize to accelerate adoption?

RBMA Comments:

a. Published findings:

There is growing research on AI's impact on clinical workflows, diagnostic accuracy, and efficiency. However, much of the literature is early-stage, limited to specific populations, or lacks large-scale deployment data.

b. Costs and benefits:

Current literature examines cost-effectiveness unevenly; many studies acknowledge potential financial benefits but do not yet quantify them with sufficient real-world evidence. Additional federally supported economic studies would help providers model total cost of ownership, reimbursement pathways, and long-term ROI.

In summary, Artificial intelligence holds substantial promise to improve the quality, consistency, and safety of radiologists' interpretations of imaging studies. AI tools can flag subtle findings, reduce perceptual errors, prioritize urgent cases, and support clinical decision-making. Yet a central challenge to encouraging innovation in this space is that current Medicare and commercial payer reimbursement systems are largely grounded in fee-for-service models that reward physician time, malpractice risk, and practice expenses rather than measurable improvements in quality or outcomes. If AI improves efficiency or reduces interpretive effort, it may paradoxically threaten reimbursement, discouraging adoption even when patient care improves.

Under existing payment structures, radiologist work relative value units (RVUs) are based on assumptions about time and cognitive effort. AI-assisted workflows that shorten reading times or reduce uncertainty could be interpreted as lowering physician work, even though the radiologist retains legal responsibility and malpractice risk. Moreover, practice costs may increase due to AI licensing, integration, validation, cybersecurity, and compliance requirements—expenses that are not well reflected in current reimbursement formulas. This misalignment creates a policy dilemma: innovation that improves quality and safety may be financially penalized.

Policymakers can address this dilemma through several complementary strategies. First, reimbursement models should evolve to explicitly recognize quality-enhancing clinical AI as a form of physician work augmentation rather than substitution. Medicare could create add-on payments or new RVU components tied to validated AI use that demonstrably improves diagnostic accuracy, reduces downstream costs, or enhances patient outcomes. Such add-on payment should be permanent and not time limited models. Second, broader adoption of

value-based payment models—such as bundled payments —would allow providers to capture the financial benefits of higher-quality, more efficient care enabled by AI.

Third, policymakers should support mechanisms to account for persistent malpractice risk. Even when AI assists interpretation, radiologists remain legally accountable. Payment policies should reflect that AI does not eliminate professional liability and may, during early adoption, increase perceived risk. Finally, federal investment in evidence generation, standards, and regulatory clarity can reduce uncertainty and lower adoption costs, making it easier for payers to reimburse AI-enabled care appropriately.

Without payment reform, AI risks becoming a victim of its own success. Aligning reimbursement with quality, accountability, and long-term value is essential to realizing AI's potential to improve radiologic care without undermining the economic foundations of medical practice.

RBMA appreciates the opportunity to provide comments on what steps can be taken to encourage adoption and use of Artificial Intelligence as a part of clinical care. RBMA remains committed to working collaboratively with HHS to advance policies that promote high-quality, cost-effective care for Medicare beneficiaries. We welcome ongoing dialogue and stand ready to serve as a resource in shaping a sustainable future for radiology and the broader healthcare system.

Sincerely,

Jamie Dyer, President RBMA

Thomas Greeson, JD, Chair Federal Affairs Committee RBMA

Christopher “Kit” Crancer, Chair Radiology Patient Action Network (RPAN) RBMA

Jessica Struve, CAE Co-Executive Director, RBMA

Linda Wilgus, CPA Co-Executive Director, RBMA